A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 April 2026

Dr Geetika Pemasiri
Goonawarra Medical Centre

Dear Dr Pemasiri,

Re: Ms Carmen Baker
DOB: 1957-08-31
Address: 4 Backhaus Avenue, SUNBURY, VIC, 3429

Summary

- **Gastroscopy biopsies negative for intestinal metaplasia.**
- **Endoscopic findings of Hill grade II gastro-oesophageal junction and mild gastritis.**
- **Normal liver function tests with resolved biochemical fatty liver.**
- **For ultrasound of the liver and gallbladder to monitor a previously identified gallbladder polyp.**
- **Surveillance gastroscopy recommended in three to four years.**
- **Action required: Please monitor the patient's thyroid enlargement and consider ENT referral for laryngeal symptoms.**

I had the pleasure of conducting a review with Carmen Baker following her recent gastroscopy. You will be pleased to know that although we were monitoring her for previously diagnosed intestinal metaplasia of the stomach, the mapping biopsies taken during this procedure were entirely negative for this condition.

While the gastroscopy showed visual evidence of gastritis, the biopsies did not identify a specific cause, such as H. pylori. We also noted a Hill grade II appearance at the gastro-oesophageal junction, suggesting some laxity, though there was no hiatus hernia or significant endoscopic evidence of gastro-oesophageal reflux disease.

We discussed Carmen's chronic cough, which primarily occurs when she is recumbent at night. She also described infrequent episodes of breathlessness that may be consistent with laryngeal

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spasms. Given the absence of significant reflux during her gastroscopy, I suspect these symptoms may be related to laryngeal hyper-responsiveness. I am aware that you are currently investigating an enlarged thyroid found on a recent CT scan, which may be a contributing factor.

Regarding her hepatic health, Carmen's liver function tests are currently normal. While there was previous concern regarding fatty liver, this appears to have resolved biochemically. There is a history of a small gallbladder polyp and a hepatic follicle that require ongoing monitoring.

Impression and Plan

I have recommended a surveillance gastroscopy be performed in three to four years. While guidelines might permit ceasing surveillance given the negative biopsies, I believe a cautious approach is appropriate in the context of her previous history of bowel cancer. I have requested an ultrasound of the liver and gallbladder to assess the stability of the gallbladder polyp. I have asked Carmen to contact my rooms once this is completed; if the findings are stable, I do not believe a further review is necessary at this time.

Regarding her cough, if her symptoms persist once the thyroid issues are managed, you may wish to consider a referral to an ENT specialist, such as Dr Amanda Richards, for further evaluation of laryngeal hyper-responsiveness. I have reassured Carmen that she does not require probiotics following any future antibiotic courses, as the microbiome is best left to recover naturally. I have advised her to continue routine annual liver function monitoring with you.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

BAKER, CARMEN : SVP 27772, HHS 167061

DOB: 31/08/1957 Age: 68 years

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Results from 27/03/2026 ..... Episode# 26P213370 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B008221 **CLINICAL NOTES** Known IM. Pylorus =? IM. Antrum = N. Incisura = N. Lesser = N. Greater curve = N. Fundus = N. Oesophagus = N vs mild reflux. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Oesophageal bx". Three mucosal fragments measuring 3-5 mm. A1. 2. Specimen labelled "Gastric Fundus bx". Two mucosal fragments measuring 3-4 mm. A1. 3. Specimen labelled "Gastric Antrum bx". Two mucosal fragments measuring 3-5 mm. A1. 4. Specimen labelled "Gastric Antrum Lesser Curve bx". Two mucosal fragments measuring 1-5 mm. A1. 5. Specimen labelled "Gastric Antrum Greater Curve bx". Two mucosal fragments measuring 2-3 mm. A1. 6. Specimen labelled "Incisura bx". Two mucosal fragments measuring 3-4 mm. A1. 7. Specimen labelled "Pylorus bx". Two mucosal fragments measuring 3-4 mm. A1. **MICROSCOPIC DESCRIPTION And DIAGNOSIS** **1. Oesophageal biopsy - A strip of unremarkable oesophageal squamous epithelium. No columnar mucosa included. No inflammatory changes are seen. 2. Gastric fundus - Normal fundic mucosa. No inflammatory changes are seen. There is no intestinal metaplasia. 3. Gastric antral mucosal biopsies - Normal antral glands with no inflammatory changes. There is no intestinal metaplasia. Negative for dysplasia. 4. Gastric antrum lesser curve - Predominantly fundic type mucosa with a few antral type glands. There is no intestinal metaplasia. Negative for dysplasia. 5. Gastric antrum greater curve biopsy - Fundic type mucosa with occasional dilated fundic gland. No intestinal metaplasia is seen. Negative for dysplasia. 6. Incisura biopsy - Gastric fundic and antral type mucosa with glandular architecture preserved. An occasional small reactive lymphoid aggregate. There is no intestinal metaplasia. Negative for dysplasia. 7. Pylorus - Gastric mucosa of some antral glandular type with preserved architecture. No intestinal metaplasia is seen. Negative for dysplasia.** Pathologist: A/Professor J. Slavin

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